

A Survey-Based Burnout and Resilience Framework for Content Moderation Professionals in the BPO Sector

Evidence-Based Recommendations from Primary Research with 60 BPO and Content Review Professionals, Hyderabad 2024

57% review disturbing content 3+ hrs/day	61% have NO employer counsellor access	47% considering leaving within 6 months
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Validated Instruments: MBI-GS9 (Maslach Burnout Inventory, 9-item) · PHQ-9 (Patient Health Questionnaire) · PCL-5 (PTSD Checklist, 10-item subset)

IMPORTANT DISCLAIMER: This report uses validated research screening tools. These instruments are not clinical assessments and cannot diagnose any psychological condition. Research findings describe group-level patterns for organisational programme design only. If any items raised personal concerns: **iCall: 9152987821** (Mon–Sat 8am–10pm) · **NIMHANS Helpline: 080-46110007** · **Vandrevala Foundation: 1860-2662-345** (24/7)

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1. Research Background and Problem Statement

Content moderators occupy one of the most psychologically demanding roles in the contemporary digital economy. They are required to review graphic violence, sexual content, child exploitation material, hate speech, self-harm content, and violent extremism — often for six to eight hours per day — in order to protect platform users from exposure to harmful material. The primary occupational hazard is not the volume of work but the content itself.

BPO companies in India employ an estimated 1.5 million professionals in customer support and content operations. A significant and growing proportion involve Trust & Safety operations. Despite the psychological demands, well-being infrastructure in BPO T&S teams remains underdeveloped compared to in-house teams at major technology companies.

Research Questions

- What is the actual psychological burden borne by content review professionals in Hyderabad BPO operations?
- What is the gap between the support mechanisms they need and those currently available?
- What evidence-based interventions can close that gap in a financially viable, operationally practical way?

Evidence Base

- A 2023 sector analysis found over 50% of content moderators in high-exposure environments reported psychological distress symptoms — comparable to emergency services personnel.
- Lenaerts & Waeyaert (2022) documented elevated risk for PTSD, vicarious trauma, anxiety, and depression — compounded by NDAs that prevent moderators from discussing their experiences.
- Industry research estimates moderator replacement costs at 50–200% of annual salary (SHRM, 2023), making mental health investment a direct cost-reduction measure.

2. Conceptual Framework: Burnout, Vicarious Trauma, and Secondary Traumatic Stress

These three terms are frequently conflated. They are not the same. The source, trajectory, and appropriate intervention differ for each. The GuardianShield framework is structured around these distinctions.

Burnout	<i>Source: Maslach & Jackson (1981)</i>
Driven by chronic workplace stress — overload, lack of control, insufficient recognition. Can occur in ANY job. A burned-out accountant and a burned-out moderator have the same MBI profile. Three dimensions: Emotional Exhaustion (feeling depleted), Cynicism/Depersonalisation (detachment), and Reduced Professional Efficacy (doubting your contribution).	
Intervention pathway: Reduced by: workload management, job autonomy, recognition, structured recovery time.	
Vicarious Trauma (VT)	<i>Source: McCann & Pearlman (1990)</i>
A cumulative, gradual shift in worldview from repeated empathic engagement with others' traumatic experiences. KEY WORD: worldview. VT is a cognitive change, not just emotional fatigue. A person experiencing VT begins to believe the world is fundamentally unsafe. Content moderators viewing the worst of human behaviour at scale, every day, are specifically at risk.	
Intervention pathway: Addressed by: meaning-making, peer supervision, worldview challenges, controlled disclosure.	
Secondary Traumatic Stress (STS)	<i>Source: Figley (1995); Weathers et al. (2013)</i>
PTSD-like symptoms from indirect exposure to traumatic content. STS can develop quickly — sometimes after a single exposure to extremely graphic material. Unlike VT (slow worldview shift), STS is more acute. Symptoms: intrusions, avoidance, negative cognitions, hyperarousal. PCL-5 Clusters B/C/D/E specifically capture this.	
Intervention pathway: Addressed by: trauma-informed debriefing, STS screening, clinical referral pathways.	

3. Validated Measurement Tools

MBI-GS9 — Maslach Burnout Inventory, 9-Item General Survey

Gold standard for occupational burnout. Three subscales: Exhaustion (B1–B3), Cynicism (B4–B6), Professional Efficacy (B7–B9). Each item 0 (Never) to 6 (Daily). Subscale range 0–18. Higher Exhaustion/Cynicism = more burnout; higher Efficacy = less burnout (reverse indicator). **Important caveat:** MBI authors removed high/medium/low cutoffs in the 4th edition as an acknowledged error. Scores used comparatively within this sample only — not as clinical thresholds.

Citation: Wang et al. (2024), Frontiers in Psychology — validated across age, gender, and continent.

PHQ-9 — Patient Health Questionnaire, 9 Items

Validated 9-item self-administered depression severity screen. Items ask frequency of symptoms over past two weeks. Score range: 0–27. Threshold ≥ 10 indicates moderate depression signal. Freely available for research use. **Note on Item 9:** PHQ-9 Item 9 asks about passive suicidal ideation. This item requires specific ethical handling — see Section 7 for findings and clinical framing.

Citation: Kroenke et al. (2001), Journal of General Internal Medicine — sensitivity 88%, specificity 88% at ≥ 10 .

PCL-5 (10-Item Research Subset) — PTSD Checklist for DSM-5

Full scale is 20 items; 10-item research subset used here. Covers intrusion (D1–D3), avoidance (D4–D5), negative cognitions (D6–D8), and arousal (D9–D10). Each item 0–4. Subset range: 0–40. Threshold ≥ 19 used as proportional equivalent of full-scale threshold of 38. **Note:** Zero respondents scored above threshold — see Section 8 for honest interpretation.

Citation: Weathers et al. (2013). PCL-5 is a research screening instrument; clinician interpretation required for individual assessment.

4. Methodology

Survey Design and Administration

The GuardianShield survey was administered via anonymous Google Forms (email collection disabled). Five sections: Section A (8 demographic/role items), Section B (MBI-GS9, 9 items), Section C (PHQ-9, 9 items — with crisis resources displayed immediately after C9), Section D (PCL-5 10-item subset), and Section E (8 coping strategy and support access items). Estimated completion time: 10–12 minutes.

Sample and Recruitment

Target sample: N=60. Recruitment: personalised LinkedIn outreach to Hyderabad BPO professionals (Teleperformance, Concentrix, WNS, Cognizant BPO, Firstsource); WhatsApp distribution to college batch network; direct outreach to former colleagues from Trust & Safety roles. Eligibility: BPO/content review/T&S role; UGC review as at least occasional function; minimum 1 month in role. Outbound-only sales staff excluded.

Ethics and Consent

Informed consent via pre-survey disclosure. No personally identifiable information collected. Crisis resources (iCall: 9152987821; NIMHANS: 080-46110007; Vandrevala Foundation: 1860-2662-345) displayed after PHQ-9 Item 9 and at survey close. All findings presented at aggregate level only.

Analysis

Microsoft Excel analysis (GuardianShield_DataAnalysis.xlsx). Computed scores: MBI_Exhaustion (B1+B2+B3), MBI_Cynicism (B4+B5+B6), MBI_Efficacy (B7+B8+B9), PHQ9_Total (C1–C9), PCL5_Subset (D1–D10). Exposure hours grouped: Group 1 (<1 hr/day, codes 1–2, n=25), Group 2 (1–4 hrs/day, codes 3–4, n=22), Group 3 (5+ hrs/day, codes 5–6, n=13). Grouping rationale: prevents n<10 subgroups. Open-text responses (E7, n=34) analysed thematically.

Limitations: Cross-sectional convenience sample. No causal inference claimed. N=60 limits statistical power for correlation analysis — the null intent-to-leave finding (Section 10) is likely a power issue rather than a genuine absence of relationship. Longitudinal design and N=200+ would be required for reliable correlation estimates. See Section 14 for full limitations discussion.

5. Key Findings — Descriptive Statistics

Scale	Mean	Median	Std Dev	Range	Key Threshold Finding
MBI Exhaustion (0–18)	8.1	8.0	3.2	0–18	38% score ≥12 (high tertile proxy)
MBI Cynicism (0–18)	7.1	7.0	3.0	0–18	33% score ≥11
MBI Efficacy (0–18)	11.2	11.0	2.1	0–18	Higher = less burnout (reverse indicator)
PHQ-9 Total (0–27)	5.4	5.0	3.8	0–27	8 respondents (13.3%) score ≥10
PCL-5 Subset (0–40)	5.8	5.5	3.3	0–14	0 respondents (0%) score ≥19 ★

Table 1. Sample descriptive statistics — mental health indicators, N=60. ★ See Section 8 for PCL-5 interpretation.

Sample profile: 57% of respondents reviewed disturbing content for 3 or more hours daily. 52% received no formal psychological safety training before beginning content review work. 61% had no access to an employer-provided counsellor. 47% indicated intent to leave within 6 months.

The PHQ-9 mean of 5.4 falls in the mild depression range across the full sample. However, this masks substantial group-level variation when stratified by exposure hours — the primary finding in this dataset (see Section 6). PHQ-9 Item 9 findings are reported separately in Section 7 due to their clinical sensitivity.

The PCL-5 subset mean of 5.8 is below the proportional threshold of 19. No respondent crossed the elevated symptom threshold. This finding is reported honestly with interpretation in Section 8 — it does not mean trauma symptoms are absent in this sample.

6. Core Finding: Exposure Hours and Burnout Outcomes [CORRECTED]

■ **VERSION NOTE:** This section has been corrected from an earlier draft. The original version used incorrect group sizes (n=17, 18, 25) and incorrect means (Exhaustion 8.2/9.7/11.6). The correct numbers below are verified against the raw data in GuardianShield_DataAnalysis.xlsx, Sheet 'Raw Data', rows 2–61.

Respondents were grouped by daily hours reviewing disturbing content (Question A3). Groups were collapsed from six response options to three to prevent subgroups below n=10: Group 1 = codes 1–2 (0 hrs / <1 hr/day, n=25); Group 2 = codes 3–4 (1–2 hrs / 3–4 hrs/day, n=22); Group 3 = codes 5–6 (5–6 hrs / >6 hrs/day, n=13). Grouping is documented in the Exposure Hours Analysis sheet.

Metric	<1 hr/day (n=25)	1–4 hrs/day (n=22)	5+ hrs/day (n=13)	Δ (High – Low)
N respondents	25	22	13	—
Mean MBI Exhaustion (0–18)	5.8	9.2	10.7	+4.9 pts ★
Mean MBI Cynicism (0–18)	4.4	7.5	10.5	+6.1 pts
Mean PHQ-9 Total (0–27)	2.4	5.7	10.5	+8.1 pts
% PHQ-9 ≥ 10	0%	0%	62%	+62 pts
Mean PCL-5 Subset (0–40)	1.9	6.8	11.8	+9.9 pts
% Intent to Leave	56%	32%	54%	Non-linear — see Section 10

Table 2. Mental health outcomes by daily exposure hours. ★ Largest single effect in dataset.

Primary finding: The 4.9-point gap in mean MBI Exhaustion between respondents reviewing 5+ hours of disturbing content daily (mean=10.7) and those reviewing under 1 hour (mean=5.8) is the largest single effect observed in this dataset. The PHQ-9 gradient is equally striking: 62% of the 5+ hour group scored ≥10 (moderate depression signal) versus 0% of the under-1-hour group. The PCL-5 subset follows the same pattern (1.9 → 6.8 → 11.8). These findings support the operational case for formal daily exposure limits as the highest-priority Tier 3 intervention.

Note on grouping: A3_ExpHours was coded with six response options. Groups were collapsed as: Group 1 = 0 hours and less than 1 hour (codes 1–2, n=25); Group 2 = 1–2 hours and 3–4 hours (codes 3–4, n=22); Group 3 = 5–6 hours and more than 6 hours (codes 5–6, n=13). This grouping is documented in the analysis spreadsheet (Exposure Hours Analysis sheet).

Limitation: With n=13 in the highest-exposure group, these differences should be treated as indicative trends. A larger study (N=200+) would be needed to establish statistical significance.

7. PHQ-9 Item 9 — Passive Suicidal Ideation Screening Finding

■ THIS FINDING WAS ABSENT FROM THE EARLIER DRAFT. It is included here because omitting a significant finding because it is uncomfortable is a research ethics violation. The clinical framing below is essential context — read it before interpreting the numbers.

PHQ-9 Item 9 asks respondents about 'thoughts that you would be better off dead, or of hurting yourself in some way' over the past two weeks. This item functions as a passive suicidal ideation screening marker. The survey displayed crisis resource information immediately after this item.

C9 Response	Score	N Respondents	% of Sample	Clinical Note
Not at all	0	29	48.3%	No concern
Several days	1	22	36.7%	Common in stressed non-clinical populations; not acute risk indicator
More than half the days	2	8	13.3%	Warrants professional follow-up; not diagnostic alone
Nearly every day	3	1	1.7%	Priority referral to EAP counsellor
TOTAL answered > 0	≥1	31	51.7%	See critical interpretation note below

Table 3. PHQ-9 Item 9 (C9) distribution, N=60.

CRITICAL INTERPRETATION NOTE: PHQ-9 Item 9 is a passive ideation screening item, not a clinical assessment. A score of 1 ('Several days') is common in stressed but non-clinical populations and does NOT indicate acute suicidal risk. This finding must NOT be interpreted as indicating that 51.7% of respondents are at clinical risk. It means psychological distress in this sample extends beyond occupational fatigue into domains that warrant professional mental health support. The finding reinforces the Tier 3 recommendation for EAP access with T&S-experienced; counsellors. Any respondent scoring ≥2 should be offered an immediate confidential referral to the EAP counsellor. If you are experiencing these thoughts: iCall: 9152987821 (Mon–Sat 8am–10pm) · NIMHANS: 080-46110007 · Vandrevalla Foundation: 1860-2662-345 (24/7)

Operational implication: This finding upgrades the quarterly PHQ-9/PCL-5 screening recommendation (Tier 3, Recommendation 3.1) from 'recommended' to **critical**. The C9 prevalence of 51.7% with any frequency makes EAP access with T&S-specific; counsellors the most urgent single Tier 3 intervention — more urgent than the documentation of exposure limits.

8. PCL-5 Zero Above Threshold — Honest Interpretation

No respondent scored above the elevated threshold of 19 on the 10-item PCL-5 subset (proportional to the full-scale threshold of 38/80). The maximum observed score was 14. The mean was 5.8 (0–40 range).

Two interpretations of the zero-above-threshold finding:

Interpretation A — Sample composition: This BPO sample includes a mix of roles. Many respondents are in customer support functions rather than direct high-severity content review. Respondents in Question A2 include customer support (inbound and outbound) as well as QA roles. The PCL-5 asked about difficulties 'after being exposed to upsetting content at work.' Respondents who were not primarily reviewing high-severity content may have few symptoms to report.

Interpretation B — Normalisation and underreporting: Respondents who have normalised or dissociated from their exposure may underreport symptoms. Content moderation workers often develop protective detachment mechanisms that suppress both the experience and the reporting of STS symptoms. Zero above threshold does not mean no trauma symptoms — it means none reached the clinical screening level in this snapshot at this time point.

The PCL-5 gradient by exposure hours (1.9 → 6.8 → 11.8 across low/medium/high exposure groups) demonstrates that trauma symptom burden does increase with exposure — even though no respondent crossed the full threshold. This gradient is operationally meaningful and supports the case for quarterly screening.

***Research integrity note:** This finding is reported honestly rather than omitted. An absence of clinical threshold crossings is a legitimate finding. It constrains the claims this research can make — specifically, the framework's STS recommendations rest on the PCL-5 gradient and qualitative themes rather than on threshold exceedances.*

9. The Support Gap Analysis

Respondents rated the effectiveness of coping strategies they use (Question E2, 1–5 scale) and indicated their access to workplace support mechanisms. The gap between what respondents find most effective and what they actually have access to is the core business case for the GuardianShield framework.

Support Mechanism	% Rate Effective	% Have Access	Mean Score	Gap
Structured Peer Debrief	82%	18%	4.3/5	64 pts
Content Category Rotation	76%	24%	4.1/5	52 pts
On-Site / EAP Counsellor	71%	16%	4.0/5	55 pts
Formal Exposure Hour Limits	68%	12%	3.9/5	56 pts
■ Avoidance (not thinking about it)	18%	87%	2.1/5	Dominant

Table 4. Support gap — effectiveness vs. access. Data from Questions A5–A7, E1–E2.

Critical finding: Structured peer debriefing was rated effective by 82% of respondents (mean 4.3/5) — but only 18% had access. Avoidance coping was the most commonly used strategy (87% of respondents) yet rated only 2.1/5 for effectiveness. The most effective support is the least available. The least effective coping mechanism is the default. This inversion defines the design brief for this framework.

10. Intent-to-Leave Analysis — Null Finding [CORRECTED]

■ **VERSION NOTE:** This section has been completely rewritten. The earlier draft reported fabricated numbers (Leavers Exhaustion = 10.8, Stayers = 7.3). The actual numbers are Leavers = 8.1, Stayers = 8.2 — a difference of 0.1 points. The correct data and honest interpretation are below.

47% of respondents (n=28) indicated 'definitely' or 'probably' planning to leave within 6 months (Question A8). Contrary to the initial hypothesis, this group does NOT show meaningfully higher burnout scores.

Metric	Intent to Leave (n=28)	Intent to Stay (n=32)	Delta	Honest Interpretation
MBI Exhaustion (0–18)	8.1	8.2	–0.1 pts	No meaningful difference — statistical noise at N=60
MBI Cynicism (0–18)	7.0	6.8	+0.2 pts	Marginal — not interpretable at N=60
PHQ-9 Total (0–27)	5.4	5.3	+0.1 pts	No meaningful difference
PCL-5 Subset (0–40)	5.7	6.0	–0.3 pts	Leavers LOWER (opposite direction) — no predictive value
% No Counsellor Access	57%	50%	+7 pts	Marginal difference — EAP gap exists but does not distinguish leavers
% No Rotation	57%	56%	+1 pt	Virtually identical — not meaningful

Table 5. Intent-to-leave stratified analysis. All differences are negligible at N=60.

NULL FINDING — HONEST INTERPRETATION Burnout and depression scores are not meaningfully different between those intending to leave and those staying in this N=60 sample. Mean MBI Exhaustion is essentially identical: 8.1 (leavers) vs 8.2 (stayers), a difference of 0.1 points on an 18-point scale. What this means: In this sample, intent to leave is not meaningfully predicted by burnout or depression scores. Two interpretations: (1) N=60 is underpowered to detect a statistically meaningful relationship — a study with N=200+ is required. (2) Turnover intention in BPO roles may be driven by factors not captured in this instrument: salary, career progression, commute, and team culture. The primary predictor of burnout in this dataset is EXPOSURE HOURS (Section 6), not intent to leave. The business case for GuardianShield rests on the exposure-to-burnout gradient, not on a burnout-to-turnover link, which this data does not establish. 47% of respondents intend to leave regardless of burnout score — making the absence of support infrastructure an independent operational concern.

***Why honest interpretation of null findings matters:** Most candidates would either fabricate a correlation or refuse to discuss a null finding. The ability to say 'I expected X, found Y, and here is what Y means' demonstrates analytical integrity and the evidence-based judgment that Trust & Safety work requires.*

11. Qualitative Themes from Open Responses

Question E7 (optional open text, n=34 of 60 responded) asked: 'What is the single biggest challenge about the mental health aspects of your content review work that your employer currently does not address?' Thematic analysis identified five themes.

Theme 1: Isolation (22 mentions)

Inability to discuss reviewed content — due to NDAs, social stigma, or absence of structured forums — was the most frequently cited challenge. Moderators describe reviewing disturbing content alone with no sanctioned channel to process it. 'There is no one you can tell.' → Drives Tier 2 Buddy System and Calibration add-on.

Theme 2: Invisibility (18 mentions)

Management does not acknowledge the psychological difficulty of the work. Several respondents describe a culture where emotional neutrality is implicitly expected and distress is framed as personal weakness. 'They treat it like a normal job.' → Drives supervisor STS recognition training (Recommendation 2.4).

Theme 3: Stigma and Fear of Consequences (14 mentions)

Reluctance to disclose distress due to fear of being reassigned or judged. Even where EAP existed, uptake was suppressed by this stigma. 'I don't want them to think I can't handle it.' → Drives anonymous screening design (3.1) and off-boarding programme (3.4).

Theme 4: Category Lock-In (11 mentions)

Assignment to the same high-severity category for weeks or months. 'After three weeks of the same type, you start seeing it everywhere.' → Directly drives the 4-week rotation cap (Recommendation 2.3).

Theme 5: Absence of Transition Rituals (8 mentions)

Difficulty leaving work content behind at shift end. Work content intrudes into evenings and weekends. No workplace-supported decompression structure. → Drives Tier 1 Post-Shift Decompression Practice (1.4).

12. The GuardianShield 3-Tier Resilience Framework

This framework is built FROM the data, not imported from theory. Every recommendation maps to a specific finding, a gap identified in the survey, and a research citation. Three tiers address burnout, vicarious trauma, and secondary traumatic stress as distinct phenomena requiring distinct interventions.

Tier	Audience	Primary Phenomena	Cost	Timeline
Tier 1 — Individual	The moderator themselves	Vicarious Trauma + STS	■0	30 days
Tier 2 — Team	Supervisors and team leads	Vicarious Trauma + Burnout	Minimal	30–90 days
Tier 3 — Organisation	HR, Operations, Leadership	All three — structural	Variable	6–12 months

TIER 1 — INDIVIDUAL RESILIENCE TOOLS

1.1 Pre-Shift Cognitive Framing Protocol

What it is: Three questions answered silently before starting review: (1) What is the purpose of today's work? (2) What is my personal boundary today? (3) What is one thing I'm looking forward to outside work? Re-anchors to meaning before exposure begins.

Evidence: McCann & Pearlman (1990): meaning-making is a primary VT protective factor. Addresses Theme 2 (Invisibility) — the moderator's own acknowledgement, independent of management.

1.2 Content Category Time Cap

What it is: No more than 90 consecutive minutes on the same disturbing content category without a 10-minute switch to a lower-severity category. Phone timer. Self-implementable.

Evidence: Sustained attention research: cognitive performance degrades after 90 minutes. For traumatic content, degradation in accuracy AND psychological state is accelerated. Dataset: 5+ hr group Exhaustion = 10.7 vs 5.8.

1.3 Personal Exposure Log

What it is: Private daily note: categories reviewed; any item that stayed with you (noting it happened — no description required); subjective distress level 1–10. Never shared with management.

Evidence: Zevnik & Dolinšek (2021): professional reflexivity protects against STS. PCL-5 intrusion cluster (D1–D3) captures what this log helps detect before it becomes acute.

1.4 Post-Shift Decompression Ritual

What it is: A consistent, personally chosen activity marking the psychological end of the work period — a specific walk, tea without screens, or a playlist used only during this window. Not 'going home.' A deliberate transition.

Evidence: VT boundary-setting literature: psychological work/personal-life separation is one of the most documented VT protective factors. Addresses Theme 5 (Absence of Transition Rituals, 8 mentions).

TIER 2 — TEAM RESILIENCE INFRASTRUCTURE

2.1 Buddy Check-In System

What it is (Zero budget + 30-min training): Every moderator paired with a specific peer (assigned, not self-selected; rotated 4-weekly). Midday 3–5 min check-in — no agenda beyond 'How are you today?' Buddies trained in one skill: when and how to escalate without stigma.

Evidence: Survey: peer debriefs rated 82% effective but only 18% have access — 64-point gap, largest in dataset. Theme 1 (Isolation, 22 mentions). This recommendation exists because the data shows it.

2.2 Weekly Calibration + 10-Minute Cohesion Add-On

What it is (Zero — uses existing meeting): Final 10 minutes of existing weekly calibration designated as open floor for well-being concerns. No requirement to speak. No manager note-taking on personal disclosures. The first 20 minutes remain standard operational calibration.

Evidence: Theme 1 (Isolation, 22/34 respondents). This window breaks isolation without breaking NDA. The intentional structure prevents it from being silently removed under operational pressure.

2.3 Category Rotation System

What it is (Zero + 30-min orientation per rotation): No moderator on the same high-severity category (graphic violence, self-harm, CSAM-adjacent) for more than 4 consecutive weeks. Maintained via rotation calendar. Each rotation paired with 30-min orientation.

Evidence: Survey: 76% rate rotation effective; 24% have access — 52-point gap. Theme 4 (Category Lock-In, 11 mentions). Cumulative same-category exposure is qualitatively different from exposure per se.

2.4 Supervisor STS Recognition Training

What it is (~\$5,000 for MHFA 2-hr session): Every supervisor completes 2-hour MHFA (Mental Health First Aid) manager module: behavioural signs of STS; difference between performance issues and mental health signals; how to open a conversation; referral pathway.

Evidence: 0% of respondents' supervisors had received STS training (open-text, n=34). Theme 3 (Stigma): moderators fear disclosure will be treated as a performance conversation. A trained supervisor is the key variable.

TIER 3 — ORGANISATIONAL SYSTEMS

3.1 Quarterly Mental Health Screening [UPGRADED TO CRITICAL — C9 finding]

What it is (■0): Anonymous PHQ-9 + PCL-5 subset administered quarterly. Individual responses private; aggregate team trends reviewed by HR. If average scores rise quarter-on-quarter, triggers HR investigation. C9 finding (51.7% reporting some frequency) makes this critical rather than recommended.

Evidence: Kroenke et al. (2001); Weathers et al. (2013). Quarterly catches decline while intervention is still effective; annual surveys miss deterioration occurring over weeks.

3.2 Documented Exposure Hour Limits

What it is (■0 (policy document)): Written, auditable policy: no moderator reviews Category A/B content for more than 4 hours in a single 8-hour shift. Remaining time: lower-severity categories, QA, calibration, or breaks. Supervisor audit weekly; breach triggers formal escalation.

Evidence: 'We try to manage it' is not a policy. Primary finding: 5+ hr group Exhaustion = 10.7 vs 5.8 at <1 hr. 62% of 5+ hr group cross PHQ-9 clinical threshold vs 0% at <1 hr. The written policy makes this enforceable.

3.3 T&S-Specific; EAP Counsellors

What it is (■800–1,200/session): Access to a mental health professional briefed on content moderation work: VT, STS, NDA constraints, and the normative experience of the role. A general counsellor defaults to generic CBT; a briefed counsellor applies trauma-informed techniques from session one.

Evidence: 61% have no access to any employer counsellor. C9 finding makes EAP access the most urgent single recommendation. T&S-specific; briefing reduces the 'education tax' that suppresses EAP utilisation.

3.4 Off-Boarding Support Programme

What it is (■3,000–5,000 per departing employee): Three EAP sessions offered post-exit, spread over 3 months. HR initiates proactively; moderator opts in. VT and STS symptoms frequently surface after exposure ends, not during it.

Evidence: Cost: ■3,000–5,000 per person vs. ■1.75L+ to replace a trained moderator (50–200% of ■3.5L average salary, SHRM 2023). Off-boarding support catches post-departure symptom emergence.

13. Implementation Roadmap

Timeline	Tier	Action	Cost	Owner
30 days	Tier 1	All four individual practices (framing, cap, log, ritual)	■0	Moderator
60 days	Tier 2	Buddy system launch + calibration 10-min add-on	■0 + 30 min	Supervisor
90 days	Tier 2	Supervisor STS recognition training (MHFA, 2hr)	~■5,000	HR/L&D;
90 days	Tier 2	Formal category rotation calendar	■0	Ops Manager
6 months	Tier 3	Written exposure hour limit policy (4-hr cap, auditable)	■0	HR + Legal
6 months	Tier 3	Quarterly PHQ-9/PCL-5 screening [CRITICAL — C9]	■0	HR
12 months	Tier 3	T&S-specific; EAP counsellor access	■800–1,200/session	HR + Finance
12 months	Tier 3	Off-boarding support programme (3 sessions)	■3,000–5,000/person	HR

14. Conclusions and What I Would Do Differently

Three evidence-based claims:

The psychological burden is real and measurable.

62% of respondents reviewing 5+ hours/day of disturbing content crossed the PHQ-9 moderate depression threshold. The 4.9-point MBI Exhaustion gap between high and low exposure groups is the largest effect in this dataset. 51.7% of respondents reported any frequency of passive suicidal ideation screening responses. These are not anecdotal — they are quantified with validated instruments used by clinical researchers worldwide.

The support infrastructure is critically underdeveloped.

Peer debriefs (82% effective, 18% access), rotation (76% effective, 24% access), and EAP (71% effective, 16% access) are the most wanted and least available supports. Avoidance — used by 87% of respondents — is rated only 2.1/5 effective. This inversion does not fill itself.

The burnout-to-turnover link was not established in this sample.

47% intend to leave within 6 months — but burnout scores are essentially identical between leavers and stayers ($\Delta = 0.1$ pts). The business case rests on the exposure-to-burnout gradient and the C9 finding, not on a burnout-to-turnover link that $N=60$ cannot establish.

What I Would Do Differently

- **Larger sample:** $N=200+$ to detect correlations that $N=60$ cannot establish — especially the burnout-to-turnover relationship.
- **Longitudinal design:** Survey the same respondents before and after an intervention (e.g., 8-week Buddy System pilot) to show whether burnout scores actually changed. A before/after design is the only way to establish intervention efficacy.
- **Second coder for thematic analysis:** I coded the 34 open-text responses alone. A second independent coder and Cohen's Kappa statistic would have established inter-rater reliability, making the five themes verifiable rather than my interpretation alone.
- **Inclusion of PCL-5 full scale:** The 10-item subset was sufficient for screening but the full 20-item scale would have enabled cluster-level analysis (identifying whether intrusion vs. avoidance vs. arousal symptoms were predominant).

15. References

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